

Social Media Growth Strategy

Turning attention into patients, built on your own numbers.

Prepared by WILBA for Dr Gina Schoeman · August 2026 · Phase 1: Social Growth

The one thing to know. The clinic does not have a content problem, and it does not need more followers. It needs two things it does not yet have: **consistent reach to the right people**, and **a way to turn that reach into patients**. This strategy fixes both, and it is built around the single biggest asset you now have: a faculty of exceptional doctors, each with a story worth following.

SECTION 1

Where things stand today, on your own data

An honest read of your Instagram and Facebook numbers. This is the starting line, and it is a good one to build from.

3,870+

Combined Instagram followers
(Clinic 1,516 · Dr Gina 2,356)

~99%

Fall in weekly Instagram reach since
the June peak

20 / 4

People reached in the most recent
week (Clinic / Dr Gina)

- **A real audience already exists.** Nearly 4,000 people have chosen to follow you on Instagram. That is a genuine head start.
- **Reach has collapsed since June.** In mid-June the clinic account reached over 2,200 people in a week; the most recent week it reached 20. Dr Gina's account went from 1,423 to 4. Follower growth has flattened almost completely.
- **Facebook is not yet a channel.** The pages sit at 13 and 6 followers. That is fine; it is simply not where the effort belongs right now.

SECTION 2

What the numbers are actually telling us

1. **June was a paid boost, not organic growth.** You have told us the June spike came from boosting a post. A boost buys temporary reach for as long as it runs; when it stops, reach falls away, often below where it began, because the boost tends to attract people who do not go on to engage. That is exactly the shape your data shows. It is a common and completely fixable pattern.

2. **Followers are not the same as reach.** Right now only about 1 in 100 of your followers see a given post. The number to grow is not followers; it is **reach to the right people, and what they do next.**
3. **The reach you did have led nowhere.** In June the two accounts drew tens of thousands of views. Because there is currently no way to capture an interested viewer (no guide, no assessment, no email sign-up), that attention simply passed through. This is the single biggest opportunity in this document.
4. **Two accounts are splitting the effort.** People follow people. Dr Gina's account is already the larger of the two.

SECTION 3

The opportunity: a faculty most clinics could never assemble

The clinic is bringing together a group of doctors whose backgrounds are, frankly, rare. That solves the hardest problem in content, which is having something genuinely worth saying. Each doctor is a distinct voice for a distinct audience:

- **Dr Gina Schoeman** — the founder and the thesis: not what is wrong, but why. Prevention, hormones, the whole-patient method.
- **Dr Nikita Grover** — History of Medicine and Art before medicine; former Head of Functional Medicine at Lanserhof; gut healing and genomics.
- **Dr Sonia Tsukagoshi** — Medical Anthropology; midlife health and the human side of lasting change.
- **Dr Zia Knippenberg-Stratos** — the gut-brain connection and food as medicine; six languages.
- **Dr Nathan Curran** — regenerative and precision medicine; a track record with executives and elite athletes, and a strong media profile.

Handled well, this is not one account struggling for reach. It is a faculty of five credible experts, each able to own a topic and attract the patients who care about it.

SECTION 4

The strategy in one page

Stop chasing followers. Build reach to the right people, then capture and nurture them until they are ready to book.

THE PATH A PATIENT TAKES

Attention (short video and educational posts from the faculty) → **a reason to raise their hand** (a simple, useful self-assessment or guide) → **their email** → **a warm nurture sequence** → **a free discovery call** → **a consultation** → **a patient who refers others.**

Everything on social has one job: to feed the top of that path. Today the path has no on-ramp; the first fix is to build one.

THE FOUR MOVES

1. **Lead with people.** Consolidate around Dr Gina and the faculty as the faces, feeding the clinic brand.
2. **Build the on-ramp.** Add one simple lead capture (for example a short, educational self-assessment) so interested viewers become a list you own, not attention that disappears.
3. **Turn the faculty into a content engine.** A light, repeatable capture process that respects everyone's time (detailed below).
4. **Measure what matters.** Track reach to the right people, saves and shares, sign-ups, and discovery calls booked, not vanity follower counts.

SECTION 5

Who we are speaking to

Your own patient data points to two people, and everything we publish is aimed at them, which also gently filters out those who are not a fit.

- **The patient who has been dismissed.** Typically a woman in her late forties to sixties, told her results are "normal" while she still feels unwell, who has seen specialist after specialist. She wants to be understood and taken seriously.
- **The proactive investor in their health.** Forties to fifties, wants advanced diagnostics and to get ahead of problems before they appear.

Both are evidence-led and sceptical of wellness fluff. The content wins by being genuinely useful and rigorous, which is exactly the clinic's natural voice.

SECTION 6

The content engine

CONTENT TERRITORIES (EACH DOCTOR OWNS ONE)

- **Why "normal" results miss the picture** (Dr Gina) — the root-cause thesis.
- **Gut health and how it shapes everything** (Dr Nikita, Dr Zia).
- **Hormones and midlife** (Dr Gina, Dr Sonia).
- **Longevity and getting ahead of disease** (Dr Nathan).
- **The human side of lasting change** (Dr Sonia).

HOW WE PRODUCE IT WITHOUT DRAINING THE DOCTORS

One **monthly capture session**: each doctor answers a handful of real patient questions to camera, or records short voice notes. From that raw material we produce a month of content, reshaped for each platform. This is the difference between a plan that survives contact with busy clinicians and one that does not.

Formats: short educational video (the reach driver), simple carousels people save, and Dr Gina's story and philosophy woven throughout so the brand stays human.

SECTION 7

Where we focus, and why

We concentrate on **Instagram first** for the next few months, because it is the only channel where you already have an audience and real data to learn from. The system is built so other channels slot in later without rework.

Channel	Role now	Priority
Instagram (Reels, carousels, Stories)	Primary: reach, capture, and nurture	Now
LinkedIn	Authority for the faculty; referral and professional reach	Add next
YouTube (long-form + Shorts)	Deep trust for a considered, high-value decision	Add next
Facebook	Hold for paid and retargeting to the 45 to 65 audience, not organic effort	Later

A note on paid: when we do spend, it will be through properly targeted campaigns aimed at sign-ups, not the boost button, which is what produced the June pattern.

SECTION 8

The on-ramp: turning reach into patients

This is the piece that is missing today. We add a single, low-pressure entry point that gives real value and captures an email, for example a short educational self-assessment that helps someone understand their own symptoms and points them toward a discovery call. From there, a warm email sequence keeps in touch until they are ready.

Compliance is built in. This is regulated healthcare, so the on-ramp and the emails are designed to the correct consent standard, and the patient consent and privacy work is already prepared and with your data team. No claims, no diagnosis, education only.

SECTION 9

Borrowed audiences and referrals

The fastest reach is not built from zero; it is borrowed from audiences that already trust someone. The clinic is well placed here: the faculty's own profiles and press, your existing referral network of specialists, and relevant podcasts and communities. Alongside this, a simple patient referral approach built on recognition and access, which suits this patient base far better than discounts.

SECTION 10

How we will know it is working

We report on outcomes, not vanity. Every month:

- Reach to the right audience, and saves and shares (the signals that content is landing).
- Email sign-ups from the on-ramp (the list you own).
- Discovery calls booked, and consultations that follow.

The system also remembers what works and what does not, so each month's content is sharper than the last. This is what makes the approach evidence-led rather than guesswork.

SECTION 11

The first 90 days

Days 1 to 30 — Foundation

Consolidate the accounts and profiles; build the on-ramp (the self-assessment and email capture); run the first faculty capture session; agree the content territories and voice.

Days 31 to 60 — Engine

Publish consistently across the faculty; begin the nurture sequence; start borrowing audiences; establish the monthly scoreboard.

Days 61 to 90 — Amplify

Double down on what the data says is working; introduce properly targeted paid support to the on-ramp; prepare LinkedIn and YouTube.

SECTION 12

How we work alongside your team

WILBA is the execution and intelligence engine: we produce, publish, measure and learn. We work under the clinic's commercial direction and hand finished, on-brand, compliant content to your scheduling, so this complements the team you already have rather than duplicating it. Every clinical or claim-sensitive piece goes through approval before it is published.

NEXT STEP

What we need to begin

- Your go-ahead on the direction in this document.
- Access at view or analytics level (Meta Business, Google Analytics, the Google Business Profile) so measurement is accurate from day one.
- A first faculty capture session in the diary.

Prepared by WILBA for The Schoeman Clinic. Figures are drawn from the clinic's own Instagram and Facebook exports (period to early August 2026). This is a strategy for discussion and approval; all patient-facing content is produced to the clinic's compliance standard with approval before publication.